



KCAC Position Statement: Kratom Use in Children

Kratom use in children under 18 years of age has been historically very low (Figure 1).¹ There are adverse events associated with kratom use in children, including agitation, hallucinations, seizures, tachycardia, and nausea (among others, Table 1).² Children could be more vulnerable to kratom adverse events, and their developing brains could be more prone to developing addictions.³ The FDA also specifies that children have lower tolerable limits of lead ingestion (interim reference levels) daily due to the negative effects of lead on brain development.⁴ This means that normal ingestion doses of many kratom products would deliver too much lead to children to be safe (Figure 2).⁴ The natural flavor of kratom powder could be a barrier for use by children.

The KCAC is aware and very concerned about marketed products that could appeal to children seeking recreational use or inadvertent ingestion based on the products' use of mascots, packaging, candy formulations (gummies, noobs, lollipops, taffy, hard candies, popping crystals, chocolate bars, cookies, brownies), shapes, colors, and flavorings. A few examples are provided in Table 2. We are aware that many of these products are not tamper resistant, could be mistaken for regular candy by small children, and are easily ingestible.

These marketing trends could increase the appeal of children looking to use substances recreationally. In 2024, 1.63 million students in middle and high school (5.9%) currently used nicotine vaping products, and 88% of them used flavored versions.⁵ Making candy or confectionary kratom products could also increase the risk of inadvertent younger child exposure. The Rocky Mountain Poison Center assessed calls from 2010-2019 regarding marijuana exposure in children < 6 years of age. Calls went from 20 per year in 2010 to more than 140 in 2019. Before 2013, there were no edible products associated with the calls, but in 2013, edible products comprised 28% of cannabis calls and have since increased to >60% of calls over the years 2016-2019.⁶

The prefrontal cortex of the brain supports decision-making, self-monitoring, and impulse control, but this part of the brain does not fully develop until the mid-20s.⁷ However, the emotional and reward-seeking centers of the limbic system develop much earlier. This helps explain why teens seek out highly stimulating activities with less regard for potential risks. Research suggests that people who start using substances at younger ages are more likely to develop substance use disorders later in life.⁷ This is why regulations for other addictive substances, such as tobacco and alcohol, limit purchases to people over the age of 21 years.^{8,9}

The KCAC believes that no kratom product should be:

- Sold to anyone under the age of 21 years, with a verification system equal to those for tobacco and alcohol to ensure compliance.
- Sold unless there are labeled warnings on the package that it is not suitable for people under the age of 21 years and that it might be habit-forming
- Marketed with a cartoon mascot on their website, product packaging, dosage form, or signage.
- Created using a formulation that resembles candy or a baked treat in its shape (teddy bears, ice cream cones, stars, footballs, faces, etc) or that contains sweeteners (be they natural or artificial), bright or pastel colors, flavorings, or scents.
- Created using any product packaging and formulation that bears any reasonable resemblance to an existing candy product that is familiar to the public as a widely distributed, branded food product such that a product could be mistaken for the branded product, especially by children.
- Able to access the product without directly involving the clerk, waitress, or seller (behind the counter, behind the display case). This means no self-service, sales from vending machines, and placement alongside other products like candy. To remove the risk of theft or purchase without showing identification.



The KCAC Recommends the Following:

A comprehensive assessment be undertaken to identify products that would be attractive to children and:

- Collate the assessment findings and report to the Food and Drug Administration and the Federal Trade Commission products deemed attractive to children for enforcement action.
- Circulate the findings to retailers so they can voluntarily stop selling the products.
- Circulate the findings to public health officials, regulatory bodies, and the general public in each state to reduce childhood exposure to these products.

The KCAC is concerned about isolate or extract products delivering high dose mitragynine, 7-hydroxymitragynine, or mitragynine pseudoindoxyl but we will address this in a separate position statement.

This position statement is open for public comment starting on Monday, October 7, 2024, and closing on Monday, November 4, 2024 [30-day comment period]. To post your comment, please click on the link: <https://bit.ly/4dy9v2d> or use the QR Code:



To learn more about the KCAC or to receive updates from the KCAC, please sign up at: www.globalkratomcoalition.org/kcac.

References:

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